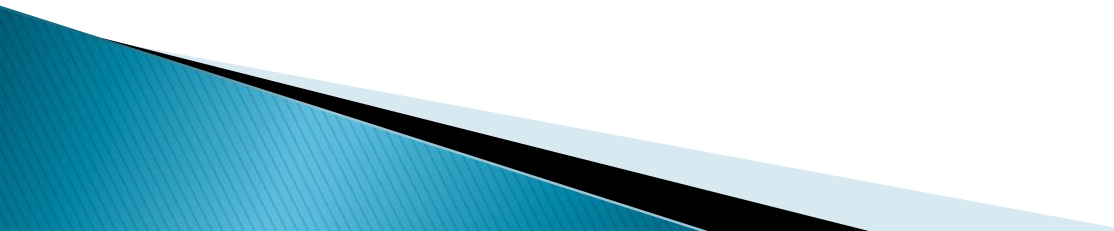


Introduction to community health nursing

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History of Community Health Nursing

- ◆ Before mid 1800s-Early Home Care with focus in Sick and Poor individual.
Orientation: curative
agencies: lay & religious orders
***St. Phoebe**
 - ◆ Mid 1800s to 1900-district Nursing with focus in Sick and poor individual
Orientation: curative/less in prevention
agencies: voluntary & some government
***William Rathbone**
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History of Community Health Nursing

- 1900-1970 public health nursing- Public Health Needs, focus in family and orientation in Curative and Prevention
agencies: government & some voluntary
***Lilliam Wald**
- 1970 to present-Community Health Nursing, focus in total community and oriented to population health, services address health promotion; illness prevention
agencies: many kinds; some independent practice
***Ruth Freedman**
- Wald was the first person to use the term public health nursing to describe the work of nurses in patients home as well as in other community settings

Basic concept:

1-Health :

Health as defined by the WHO is “*A state of complete physical mental and social well-being and not merely absence of disease or infirmity*”.

▶ **2-Disease:**

If we divide the word disease, it will be: dis-ease which means being not at ease, or in other words it denotes a state of deviation from the normal status.

▶ **3- Community :**

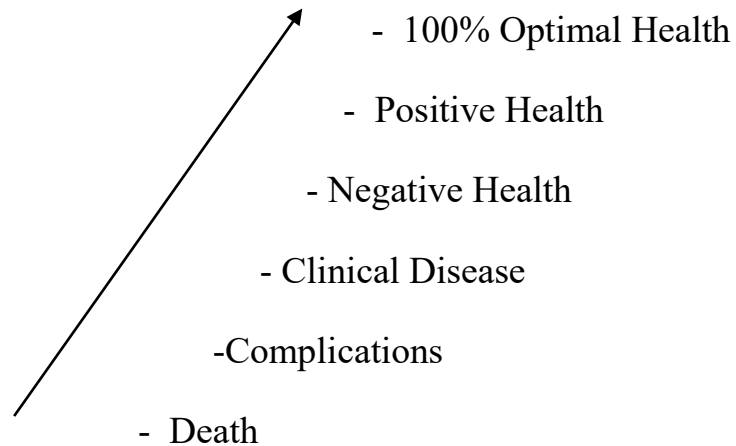
the residents of a geographic area

4- Community health: includes in state of health of the members of the community, the problems affecting their health and the health care provided for the community.

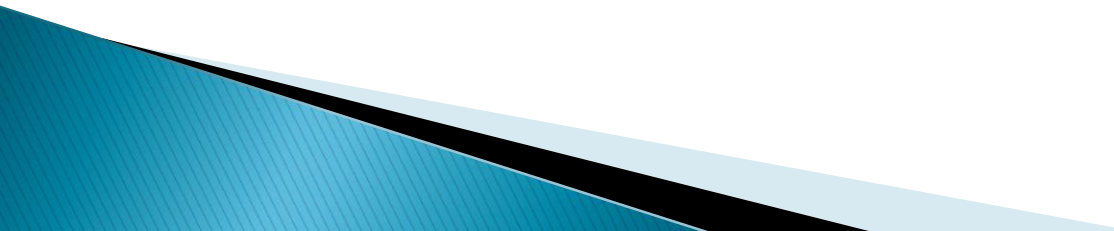
▶ **4- Community Health Nursing:** Is defined as a field of nursing practice that blends knowledge and skills from both the nursing and the public health sciences and applies them to the promotion of optimal health for the total population.

▶ **5- Community Health Nurse:** Is Registered Nurse that work in conjunction with numerous Health District programs, such as case management, and many community partners. She provides home follow up care, immunizations for homebound patients, health education and referral of clients to appropriate agencies for assistance.

A spectrum of health divides it into stages:



The Characteristics of the Community:

- ▶ **1-Defining the Community Denominator:**
 - ▶ **2-Population Demography:**
 - ▶ **3-Health Agencies and Implemented Programs:**
- 

community denominator

- 1- *Incidence* measures the rate of occurrence of new cases (the number of new cases during a period of time)
- 2- *Prevalence* is the number of cases existing in a defined population at a given point of time

Narrowing the Denominator toward Problem-Based Subpopulations

- ▶ 1-School Children
- ▶ 2-Vulnerable groups:
- ▶ 3-Work site:
- ▶ 4-Hospitals and Health care units:

2-Population Demography

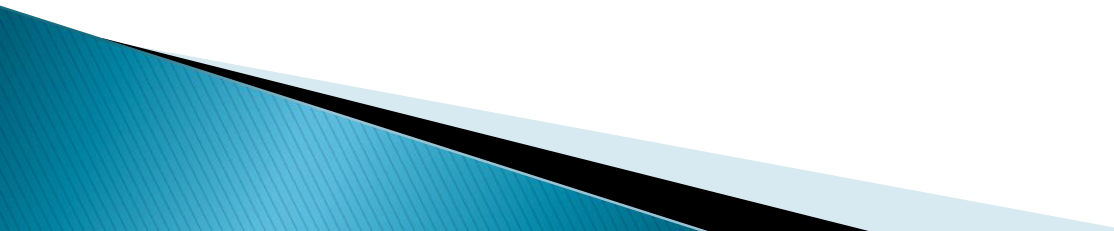
a-Age: it is a basic variable that is closely related to disease patterns, but it is difficult to ascertain its accuracy.

b-Sex: It is important since it reflects different physiological and behavioural patterns in the two sexes.

c-Marital Status: It can be important as it may reflect an important social role and life style.

d-Education and occupation: these variables will be responsible for people behaviour, attitudes as well as their exposure to some health hazards.

e-Other Variables: such as parity for females, religion, social class etc may help to explain the health status and problems prevailing in the community.



3-Health Agencies and Implemented Programs:

- ▶ **As a primary health care:**

Rural areas:

1-A rural health unit is established for every village with a population of 4000 individuals.

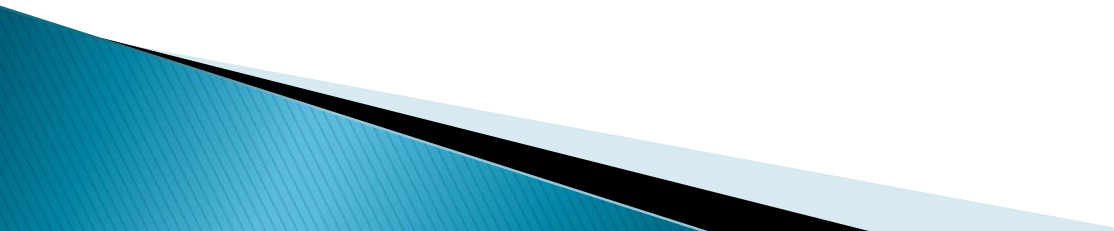
2-A rural health unit, for a collection villages with a total population of 5000 individuals and at a distance of not more than three kilometres, is established in the central village.

3-For separate village which cannot be gathered and with a population size less than 3000, an outpatient clinic is established where a doctor from the nearest PHC unit can be visited 2 to 3 times per week.

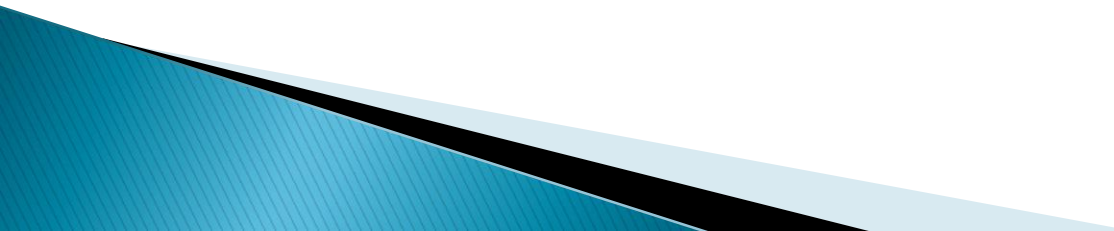
Urban areas:

- ▶ 1-Maternal and Child health care centres and Family Planning.
 - ▶ 2-School health units
 - ▶ 3-Control of infectious diseases and Food sanitation
 - ▶ 4-Curative and dental care by a general practitioners.
 - ▶ 5-Emergency services.
 - ▶ 6-Health Education
 - ▶ These services are delivered through Urban centres. Later on the family doctor policy is now implemented.
- 

Identify and prioritize health problems of our community

- ▶ **Frequency of the condition**
 - ▶ **Level of morbidity (severity)**
 - ▶ **Level of mortality (fatality)**
 - ▶ **Effectiveness and feasibility of intervention**
 - ▶ **Cost of intervention.**
 - ▶ **Time constraint.**
- 

Factors that affect the health of any community:

- 1) Social and cultural factors
 - 2) Physical factors
 - 3) Community organization
 - 4) Individual behavior
- 

1- Physical factors

✓ **Geography:**

- ▶ A community health problem can be directly influenced by climate (humidity, temperature,).

✓ **Environment:**

- ▶ The quality of our environment is directly related to the quality of our stewardship over it. (Water, soil, and air).

✓ **Community size:**

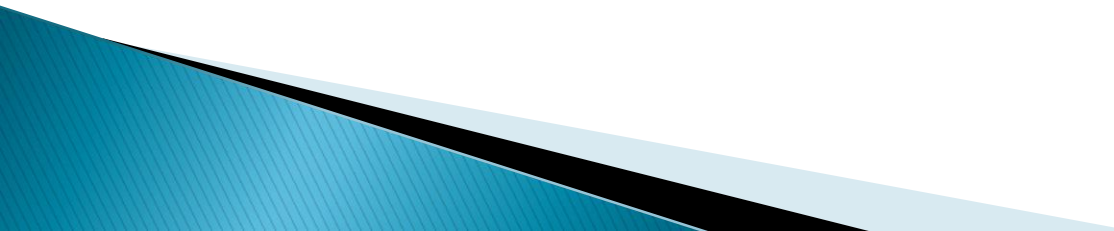
- ▶ The larger the community the greater its range of health problems and the greater its health resources.

✓ **Industrial development:**

- ▶ Industrial development, like size, has either positive or negative effects on the health Status of a community.

Social and cultural factors:

- ✓ **Social factors** are those that arise from the interaction of individuals or groups within the community
- ✓ **cultural Factors:**
 - ▶ Beliefs, traditions: The beliefs of those in a community about such specific health behaviors as exercise and smoking. The tradition of specific ethnic groups can influence the types of food, restaurants, and *bars* in community.
 - ▶ Economy: Both national and local economy can affect the health of a community through reduction in health and social services.

- ▶ Politics: Those who happen to be political offices; either nationally or locally, can improve the health of their community by the decisions they make.
 - ▶ Religion: A number of religions have taken a position on health care e.g. address moral e.g. prevent AIDS.
 - ▶ Socio –Economic status (S.E. S): Segments of the community with the lowest socio-economic status also have the poorest health and the most difficulty in gaining access to health care.
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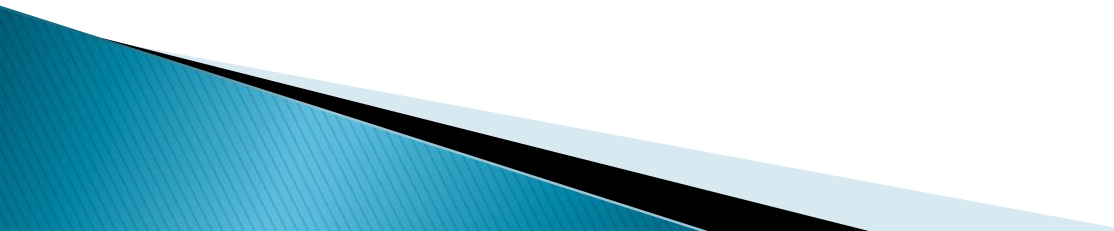
1. Community organization

▶ The way in which a community is able to organize its resources directly influences its ability to intervene and solve problems, including Health problems community organization is the method of intervention whereby individuals, groups and organization engage in planned action to influence social problems.

1. Individual behavior

▶ The behavior of the individual member of a community contributes to the health of the entire community.

Characteristics of community health nursing:

- 1) It is a field of nursing.
 - 2) It combines public health with nursing.
 - 3) It is population focused.
 - 4) It emphasizes prevention, health promotion, and wellness.
 - 5) It promotes client responsibility and self-care.
 - 6) It uses aggregate measurement and analysis.
 - 7) It uses principles of organizational theory.
 - 8) It involves inter professional collaboration.
- 

Role of the community health nurse

1. Direct care provider

This role involves the direct delivery of care, performing tasks or skills for which the nurse has been trained e.g. (client assessment, taking vital signs, giving medication, changing dressing and inserting canula or catheter)

2. Educator

Health education is an important component of quality nursing care. Health education is the most significant role of community nurse, teaching individuals, families and groups about maintenance of health threats to health, and relevant lifestyle choices that affect health.

3. Counselor

The nurse here listens to clients and their families encourage them to explore issues and options, and enable them to manage their personal situation, Assistance with identification of problems and possible solutions and guidance through the problem solving process.

4. Advocate

In community settings the nurse's role as an advocator is vital. Because they often work with vulnerable population. **For example**, the school nurse identifies a source of funding to pay for eyeglasses for the child whose parents cannot afford them.



5- Manager

Nursing care in both hospital and community settings involves management of nurses time ,limited resources ,other personnel, and program organization and coordination, as well as the management of the clients care.

6. Collaborator

The nurse must work with other health providers to deliver the most comprehensive, effective, cost conscious care possible. For example, caring for stroke victim may involve collaboration between physician, nurse, physical therapist, occupational therapist, speech therapist, and possibly others in hospital and community settings.

7. Role model Role model is person who demonstrates an action or behavior that is learned by others. Nurse serves as role model to clients, their care givers, and other health professionals.

8. Researcher Nurse should critically review research findings for merit and applicability to practice and determine care accordingly.

9. Leader

Leadership refers to the ability to influence the behavior of others. The nurse in community settings may assume the role of a leader with clients, other health care providers, public officials, local leaders, or employers.

